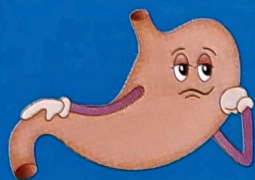


CHAPTER

1



Agnimandya- Ajirna, Anaha, Adhmana, Atopa

Learning Objectives

- ⇒ To recall the physiology of Agni
- ⇒ To identify the role of Agni in the manifestation of Agnimandya, Ajirna, Anaha, Adhmana, Atopa
- ⇒ To understand the etiological factors of Agnimandya, Ajirna, Anaha, Adhmana and Atopa mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka of Agnimandya etc diseases based on the progression of the disease
- ⇒ To learn the concepts of above mentioned diseases with the lens of physiology, pathology, physics, chemistry and biology
- ⇒ The outcome of the whole chapter is to identify the morbidities in accordance with principles of Ayurveda pathology
- ⇒ This is learnt under the domain of CK (cognitive/ knowledge); topic is MK (Must know); learnt under lecture with PPT method; assessment is done on quiz for Anaha; descriptions of Adhmana and Atopa, assessment is done in internal assessment
- ⇒ The concepts Anaha, types of Anaha should be known to the students where as for the descriptions of Adhmana and Atopa, the students should be in a position to describe know how part.

Introduction

In Ayurveda, it is being postulated that most of the *Nija* variety of diseases which manifest in the body are due to abnormality of Agni (digestive fire). In most of the occasions, this abnormality occurs as a result of abnormal diet pattern following by human beings. That is the reason, *Atanka Prakasha* (second paper of *Roga Nidana* and *Vikriti Vigyan*) is started with the topic of *Agnimandya*, *Ajirna*, *Anaha*, *Adhmana* and *Atopa*. Before learning the concepts of these pathological entities, it is better to know about the physiological and pathophysiological aspects of the diseases.

Agnimandya (see Figure 1) is a spectrum of dysfunctions where we can observe the poor functioning of Agni in an individual. In most of the situations, *Agnimandya* and *Mandagni* are used synonymously. Actually, both are distinct clinical conditions. *Mandagni* is the condition which is seen in the people who have *Kapha Prakriti* and indulge in unwholesome food and drinks. Whereas *Agnimandya* is the clinical condition which can be seen in any of the vitiation of *Dosha* either *Vata* or *Pitta* or *Kapha* which depends on the exposure to etiological factor. One thing which is common both *Agnimandya* and *Mandagni*,

there will be poor functioning of Agni. The main *Srotas* afflicted will be *Annavaha Srotas*, but in due course of time, we can observe the involvement of either *Purishavaha* or *Pranavaha Srotas* depending upon the nature of movement of vitiated *Dosha* in the *Mahasrotas*. The involvement of these two *Srotas* can be said as whole *Mahasrotas* contains part of *Pranavaha Srotas*, *Annavaha* and *Purishavaha Srotas*.

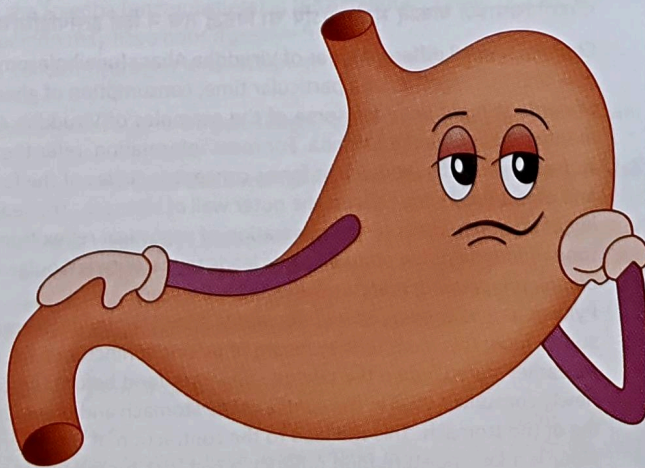


Figure 1: Pratyatma Lakshana of Agnimandya

- Frequently sleeping¹⁶ in day time after food intake.
- Increased frequency of bathing¹⁷.
- Increasingly spending time in tub bath¹⁸.
- Frequently drinking water¹⁹ in between the consumption of morsel during food intake.
- Frequent consumption of stale or rotten food.

Above etiological factors will lead to the vitiation of *Vata* etc *Dosha* and finally resulting in poor functioning of *Agni*.

Samprapti Ghataka

- **Dosha:** *Kapha* or *Pitta* or *Vata*
- **Dushya:** *Rasa*
- **Srotas:** *Annavaha, Pranavaha, Rasavaha Srotas*
- **Srotodushti Prakara:** *Sanga* and *Vimargagamana*
- **Agni :** *Jatharagnimandya*
- **Ama:** *Jatharagnimandyaajanya*
- **Udbhava Sthana:** *Amashaya*
- **Sanchara Sthana:** *Koshtha*
- **Vyakta Sthana :** *Koshtha*
- **Vyadhi Svabhava :** *Ashukari*
- **Rogamarga :** *Abhyantara*
- **Sadhyasadhya :** *Sadhy* to *Krichrasadhy*

As stated in the introduction, *Mandagni* is the condition which is usually seen in the persons who are having *Kapha Prakriti*. It doesn't mean that the state of *Mandagni* is present always in *Kapha Prakriti* individuals. It depends on the nature of food intake or quality of food intake etc. However, it is certain that when *Kapha Prakriti* individuals consume *Kapha* vitiating factors, *Mandagni* is common phenomenon, where the person is having digestion very slowly, even the substances light to digest take lot of time to get digested.

Agnimandya is the progressive pathological mechanism

where the person will have heaviness of the abdomen, heaviness of the head, cough, breathlessness, excess salivation, vomiting and sluggish movements of the body. Literally speaking, actual mechanism of the *Agnimandya* due to *Vata* or *Kapha Dosha* vitiation is not explained clearly. In the context of *Vataja Jvara* and *Kaphaja Jvara*, there is a term *पिधायामिमुपहत्य* is explained which is an indication of poor digestive function in case of *Vata* or *Kapha Dosha* vitiation. In *Pitta* dominance clinical condition, *Agnimandya* occurs due to the increase of *Drava* quality of *Pitta*. Normally, *Drava* quality will be reduced during digestion and this phenomenon is responsible for the increase of *Tikshna* quality which increases appetite of a person. This *Tikshna* quality easily facilitates the process of digestion.

- *Agnimandya* and *Mandagni* are used synonymously most of the times. *Mandagni* is usually seen in *Kapha Prakriti* individuals whereas *Agnimandya* can be observed in individuals irrespective of the *Prakriti* of the individuals. *Agnimandya* can be seen in individuals depending on the exposure to the aetiological factors, which leads to the abnormal increase of either *Vata* or *Pitta* or *Kapha* in the body.
- *Ahara Parinama Krama* is explained in brief in this chapter. *Pranavata* will lead to the movement of food substances from the mouth to the *Amashaya*. In the *Amashaya*, ingested food stays for a certain period of time. *Ushma*, *Kleda*, *Sneha* will act on the ingested food. After a certain period of time, partially digested food substances will be taken to the distal parts by *Samana Vata* which is responsible for the separation of nutrients and metabolites. Further, *Samana Vata* helps in the absorption of nutrients and expulsion of metabolites (undigested food residues) further into *Pakvashaya*. *Apana Vata* will take up the function of expulsion of undigested food residues to the exterior in due course of time.

leads to *Abhishyanda* (obstruction to *Srotas*), it becomes much heavy for digestion. That is why, when an individual has the habit of excess beaten rice consumption, that will lead to poor digestive function.

- 16 While analysing this etiological factor, we have to concentrate on two different aspects: Sleeping during day time and time of sleep i.e. after consumption of food in the noon time.
 - Sleeping during day time, the person will have relaxation of most of the muscles of the body. This includes the relaxation of tone of airway dilator muscles results in collapse in the upper airways during sleep. Because of this mechanism, the person will have increased mouth breathing which results in snoring.
 - When an individual goes for sleep soon after the lunch, the body will have increased vascularity towards gastrointestinal compartment and there will be poor blood circulation to the nervous system and because of this reason, the person will have natural tendency of sleepiness during day time.
 - During sleep, there will be parasympathetic nervous system stimulation, this will lead to the enhanced alimentary secretion and there will be poor muscular activity and this will cause poor digestion of food.
- 17 The nature of water used for bathing either cold or hot water will have an impact on regulation of fluid compartments inside the body. If the person uses hot water for frequent bathing, it leads to intense vasodilatation of skin blood vessels. This will lead to the increase of the rate of heat transfer to the skin upto 8 folds. This leads to the decrease fluid of intravascular compartment, thereby there will be decreased secretion in the gastrointestinal compartment and thus the person will have abnormal digestion and assimilation.
- 18 In this also, the mechanism is the same as that of frequent bathing of warm water.
- 19 Frequent drinking of water in between meals is responsible for the dilution of hydrochloric acid and this will lead to the dilution of *HCl* and thus it will lead to poor activation of pepsinogen and thus the person will have improper digestion.

- ❖ Agnimandya and Ajirna are manifested in the individuals as abnormal cycles. Agnimandya will lead to the occurrence of Ajirna and Ajirna will lead to the manifestation of Agnimandya and the cycle continues.
- ❖ Kashyapa mentioned the aetiological factors of Agnimandya in the context of Amlapitta in Khila Sthana of Kashyapa Samhita.

Ajirna

Introduction

Symptoms of poor digestion

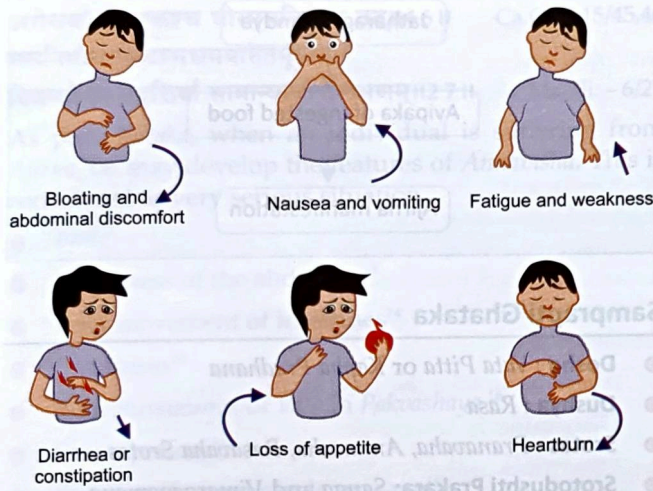


Figure 2: Lakshana of Ajirna

Ajirna is the spectrum of the diseases where digestion of ingested food doesn't occur at appropriate time. Digestion is a very essential phenomenon that occurs when macronutrients are converted to micronutrients which can be utilised for the supplementation of depleted body nutrients.

Due to the disturbance in digestive function, the person may be presented with *Ajirna*. When we speak of *Agni Vyapara*, first there will be action of *Jatharagni*, followed by the action of *Bhutagni*, *Dhatvagni* and *Bhutagni* respectively.

This can be categorised under *Doshabala Pravritta Vikara* of *Adhyatmika Vikara*. This is a disease of *Annavaha Srotas* with the involvement of *dysfunctioning Jatharagni* responsible for the formation of *Ama* in the *Annavaha Srotas*. As far as

the type of *Srotodushti* involved, *Sanga* and *Vimargagamana* will be happening during the progression of the disease. Depending upon the movement of *Dosha*, involvement of *Dhatu* in the pathogenesis, we can observe the affliction of either *Pranavaha Srotas* or *Purishavaha Srotas* or *Rasavaha Srotas* in due course of time sooner or later.

सर्वमजीर्णं त्रिदोषजम्।

Madhu, Ma.Ni. – 6/5,6

Though, all *Ajirna* are considered to be *Tridoshaja* in manifestation, depending upon the *Dosha* dominance in disease causation, progression and clinical manifestation, *Ajirna* from single *Dosha* vitiation is considered in progression of *Ajirna*.

न जीर्यति सुखेनात्रं विकारान् कुरुतेऽपि च।

तदजीर्णमिति प्राहुस्तन्मूला विविधा रुजः॥

Gananath Sen

As per veteran *Ayurveda* scholar, Gananath Sen, *Ajirna* is a condition where a person cannot able to digest the ingested food and is responsible for various kinds of discomfort and various kinds of disease manifestation.

Before learning the description of *Ajirna*, we should know about the clinical features which indicate proper digestion of ingested food. The features are mentioned in *Madhava Nidana* as follow:

Jirnahara Lakshana

उद्गारशुद्धिरुत्साहो वेगोत्सर्गो यथोचितः।

लघुता क्षुत्पिपासा च जीर्णाहारस्य लक्षणम्॥24॥

Ma.Ni. – 6/24

- ❖ **Udgara Shuddhi¹** – clear eructation without taste perception
- ❖ **Utsaha²** – able to do any kind of activities
- ❖ **Vegotsarga³** – proper evacuation of different Vega i.e. *Vata*, *Vit* and *Mutra* (flatus, faecal matter and urine respectively) at proper time

G
K

- 1 The presence of clear eructation indicates the complete passage of food substances from Amashaya to the distal parts for digestion. When the person is developing the eructation similar to food substance consumed indicates improper mixing of digestive juices in the stomach. The presence of sour eructation indicates the mixing of hydrochloric acid in the stomach and reversal of movement of food substances in the upper alimentary canal.
- 2 The presence of Utsaha in the person indicates transformation of macromolecules into micro molecules and complete assimilation of nutrients from the alimentary canal. Otherwise, the ingested food which contain macromolecules, they may enter into lower alimentary canal as macromolecules or intermediate molecules. These molecules will move slowly in the lower alimentary canal due to heavier weight and thus the person may have lassitude.
- 3 It is being said in western medicine regarding transit time in different parts of alimentary canal. Transit time in the oesophagus is in seconds, whereas transit time in the stomach is around 2 hours, in the small intestine, the transit time is 5 to 6 hours and in the large intestine, it is 1 to 2 days. In this context, Vegotsarga refers to the movement of the consumed food substances in different parts of alimentary canal at appropriate time.

- ❖ **Laghuta⁴** – lightness of the alimentary canal
- ❖ **Kshut Pipasa⁵** – proper appearance of hunger and thirst

Ajirna Nidana

अत्यम्बुपानाद्विषमाशनाच्च संधारणात्स्वप्नविपर्ययाच्च।
कालेऽपि सात्त्यं लघु चापि भुक्तमन्नं न पाकं भजते नरस्य॥7॥
ईर्ष्याभयक्रोधपरिलुतेन लुब्धेन रुग्दैर्न्यनिपीडितेन।
प्रद्वेषयुक्तेन च सेव्यमानमन्नं न सम्यक्परिपाकमेति॥8॥
मात्राऽप्यभ्यवहतं पथ्यं चान्नं न जीर्यति।
चिन्ताशोकभयक्रोधदुःखशय्याप्रजागरैः॥9॥
अनात्मवन्तः पशुबद्धुञ्जते येऽप्रमाणतः।
रोगानीकस्य ते मूलमजीर्णं प्राप्नुवन्ति हि॥1॥4॥

Ma.Ni. – 6/7-9

Ma.Ni. – 6/14

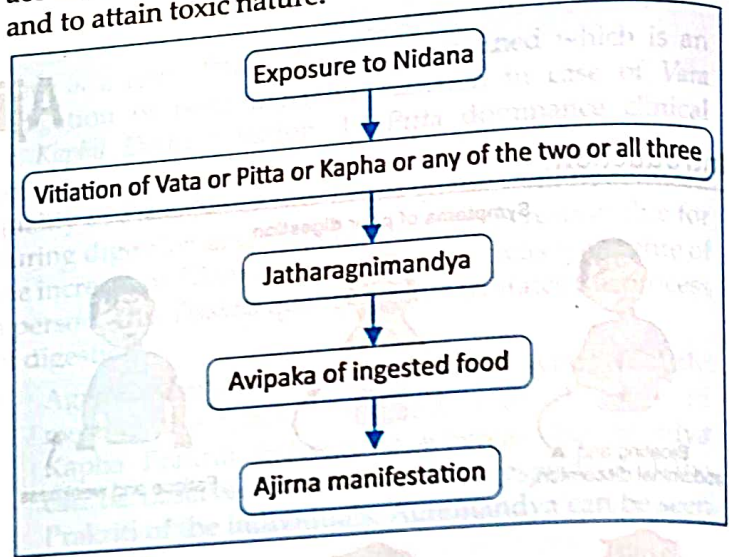
- ❖ Excess drinking of water⁶
- ❖ Consumption of food at improper time and quantity⁷
- ❖ Suppression of natural urges⁸
- ❖ Abnormal sleep patterns⁹
- ❖ When the person is having psychological distress such as jealousy, fear, anger and in these situations, even the person is taking the food at appropriate quantity and time
- ❖ If the person is having aversion to take food substances¹⁰
- ❖ Consuming the food without considering the quality or quantity of food like lower animals such as cow¹¹

Samprapti of Ajirna

दुष्यत्यग्निः, स दुष्टोऽन्नं न तत् पचति लघ्वपि।
अपच्यमानं शुक्तत्वं यात्यन्नं विषेरूपताम्॥4॥4॥

Ca.Ci. – 15/44

Patients who have developed the dysfunction of Agni are not in a position to digest the ingested food substances which are light to digest. This will lead to undigested and accumulated food in the *Amashaya* to become sour taste and to attain toxic nature.



Samprapti Ghataka

- ❖ **Dosha** : Vata Pitta or Kapha Pradhana
- ❖ **Dushya** : Rasa
- ❖ **Srotas** : Pranavaha, Annavaha, Rasavaha Srotas
- ❖ **Srotodushti Prakara** : Sanga and Vimargagamana
- ❖ **Agni** : Jatharagnimandya
- ❖ **Ama** : Jatharagnimandyaajanya
- ❖ **Udbhava Sthana** : Amashaya
- ❖ **Sanchara Sthana** : Koshtha
- ❖ **Vyakta Sthana** : Koshtha

G
K

- When the person feels lightness of the alimentary canal, then it indicates proper digestion occurring in the alimentary canal. When the person is having indigestion, that will lead to the heaviness of the alimentary canal as well as body parts.
- Appearance of appetite and thirst at appropriate time indicates the metabolic activities happening at appropriate pace and time.
- When an individual consumes excess amount of water at a particular time is responsible for the dilution of hydrochloric acid and gastric juices. When there is dilution of gastric juices, that will lead to the improper digestion of ingested food, protein rich food substances in particular. Because of the dilution of hydrochloric acid, activation of pepsinogen into pepsin is hampered which is necessary for the digestion of proteins; protection of gastric wall from microbes which enter via ingested may be hampered.
- When an individual follows this kind of eating pattern, there will be irregularities in amounts of secretion of gastric juices, thus the person may have irregular digestion and assimilation of food.
- Let us take the example of Purisha (faecal matter) and Adhovata (flatus) Vegadharana. When an individual suppresses these urges, that will abnormally increase the Apanavata in the body, which leads to the reverse movement of Vata in the body. This event will lead to the abnormal increase of Samanavata in the body. Samanavata is considered to be one of the six factors responsible for the proper digestion of food (Ahara Parinamakara Bhava). When Samanavata is disturbed, this may have an impact on Ushma, Kleda, Sneha, Kala and Samayoga of Ahara Parinamakara Bhava, leading to dysfunction of Agni. If the person still does suppression of urges, that may lead to the abnormal functioning of Pranavata (reversal of Pranavata).
- Abnormal sleeping patterns include avoiding the sleep at night time and sleeping during day time. When an individual sleeps during day time will lead to the stimulation of parasympathetic nervous system. This activation will lead to the excess secretion of juices in the alimentary canal. This may dilute the whole digestive process. When an individual awakens at night time, this will lead to the activation of sympathetic nervous system. This activation will lead to hampered secretion of juices in the alimentary canal which further disturbs the digestion process.
- Emotional stimuli frequently increase the interdigestive gastric secretion (highly peptic and acidic) to 50 ml or more per hour. Thus, extreme stressful situations are the frequent causative factors of gastric mucosal damage leading to irregular digestion.
- This kind of food consuming pattern is responsible for the stretching of the stomach wall which will lead to improper mixing of gastric juices with ingested food. Thus, the person may have indigestion of ingested food.

- ✳ **Rogamarga** : *Abhyantara* and *Bahya*
- ✳ **Sadhyasadyata** : *Sukhasadhyata* to *Krichrasadhyata*
- ✳ **Vyadhyavastha** : *Ashukari* or *Chirakari*; *Saama* or *Nirama* depending upon the duration of the illness and nature of *Dushya* involvement in pathogenesis

Samanya Lakshana of Ajirna

तस्य लिङ्गमजीर्णस्य विष्टम्भः सदनं तथा।
शिरसो रुक् च मूर्च्छा च भ्रमः पृष्ठकटिग्रहः॥१५॥
जृम्भाङ्गमर्दस्तृष्णा च ज्वरश्छर्दिः प्रवाहणम्।
अरोचको विषाकश्च घोरमन्नविषं च तत्॥१५॥६॥ Ca.Ci. – 15/45,46
ग्लानिगौरवविष्टम्भभ्रममारुतमूढताः।
विबन्धो वा प्रवृत्तिर्वा सामान्याजीर्णलक्षणम्॥१५॥७॥ Ma.Ni. – 6/27

As per *Charaka*, when an individual is suffering from *Ajirna*, he may develop the features of *Annavisha*. This is considered as very serious situation.

- ✳ **Fatigue**¹²
- ✳ **Heaviness of the abdomen**¹³
- ✳ **Poor movement of intestines**¹⁴
- ✳ **Giddiness**¹⁵
- ✳ **Poor movement of Vata in Pakvashaya**¹⁶
- ✳ **Constipation**¹⁷
- ✳ **Excess unformed stools passage**¹⁸
- ✳ **Headache**¹⁹
- ✳ **Altered state of consciousness**²⁰
- ✳ **Stiffness of the back**
- ✳ **Stiffness of the waist region**²¹

- ✳ Excessive yawning
- ✳ Thirst²²
- ✳ Raised body temperature²³
- ✳ Vomiting²⁴
- ✳ Excessive straining for defaecation²⁵
- ✳ Tastelessness
- ✳ Impaired digestion

Sankhya Samprapti of Ajirna

आमं विदग्धं विष्टब्धं कफपित्तानिलैस्त्रिभिः।
अजीर्णं केचिदिच्छन्ति चतुर्थं रसशेषतः॥१५॥
अजीर्णं पञ्चमं केचिन्निदोषं दिनपाकि च।
वदन्ति षष्ठं चाजीर्णं प्राकृतं प्रतिवासरम्॥१६॥ Ma.Ni. – 6/5,6

Types of Ajirna:

4 types as per Sushruta: *Amajirna*, *Vidagdhajirna*, *Vishtabdhajirna*, *Rasasheshajirna*

6 types as per Madhavakara

1. *Amajirna*
2. *Vidagdhajirna*
3. *Vishtabdhajirna*
4. *Rasasheshajirna*
5. *Dinapaki Ajirna*
6. *Prakrita Ajirna*

Amajirna Lakshana

तत्रामे गुरुतोत्क्लेदः शोथो गण्डाक्षिकूटगः।

- 12 This is attributable to the poor digestion and assimilation of nutrients from the alimentary canal. Glucose, sodium, potassium and calcium ions are required for most of the activities in the body. If there is deficiency in absorption of these substances, the person may have fatigue with slight amount of physical activities.
- 13 When an individual is having poor digestive fire and he has consumed food substances, most of the food substances are macromolecular and non-absorbable form. Because of the poor digestive fire, these macromolecular substances which are heavy in nature, move slowly in the alimentary canal. This will lead to the clinical presentation of heaviness of the abdomen.
- 14 When macromolecular substances enter the small intestine and large intestine, will lead to the poor intestinal motility owing to their macromolecular structure.
- 15 This can be attributable to poor cerebral blood flow or hypoglycemia or electrolyte imbalances.
- 16 This can be attributable to the presence of macromolecules in the large intestine. Due to the heavier nature, these macromolecules move slowly in the large intestine.
- 17 When there is slower movement of food residue in the large intestine, that will lead to the excess absorption of the water from the large intestine. It is well known that faecal matter in the rectum will have 75% of the water and 25% undigested food residue. When there is poor motility, this will lead to absorption of water portion from large intestine as well as rectum.
- 18 When there is excess amount of water enters the large intestine, more amount of water is going to get eliminated from the anus. Thus, the person may have unformed stools passage. This symptom will be absent in the people presenting with constipation.
- 19 When there is excess amount of water enters the large intestine, more amount of water is going to get eliminated from the anus. Thus, the person may have unformed stools passage. This symptom will be absent in the people presenting with constipation.
- 20 This can be attributable to either electrolyte imbalances or due to hypoglycemia or hypovolemia.
- 21 This can be attributable to the referred pain that develops as a result of similar innervation of back and abdominal viscera.
- 22 This is attributable to poor water absorption from the alimentary canal leading to hypovolemia.
- 23 This can be attributable to the increased metabolic activity which occurs as a result of poor digestive activity in the alimentary canal.
- 24 This is attributable to prolonged closure of pyloric antrum. This is one of the commonest occurrence in case of *Adhyashana* or *Vishamashana* nature of food intake.
- 25 This can be attributable to the presenting complaint of constipation.

उद्गारश्च यथाभुक्तमविदग्धः प्रवर्तते॥१०॥

Ma.Ni. - 6/10

- This is manifested due to the vitiation of *Kapha Dosha* in the body
- Heaviness of the body parts
 - Nausea
 - Swelling near the temporal region
 - Swelling around the orbital region
 - Eructation having the taste as the same regarding the dominance of food when he consumes food

Vidagdha Ajirna Lakshana

विदग्धे भ्रमर्तुमूर्च्छाः पित्ताश्च विविधा रुजः।

Ma.Ni. - 6/11

उद्गारश्च सधूमाग्नः खेदो दाहश्च जायते॥११॥

- This is manifested due to the dominance of *Pitta Dosha*.
- Giddiness
 - Thirst
 - Altered state of consciousness
 - Different kinds of discomfort
 - Belching having sour taste
 - Sensation of smoke coming out through mouth
 - Sweating
 - Burning sensation

Vishtabha Ajirna Lakshana

विष्टब्धे शूलमाध्मानं विविधा वातवेदनाः।

Ma.Ni. - 6/12

मलवाताप्रवृत्तिश्च स्तम्भो मोहाङ्गपीडनम्॥१२॥

- This is manifested due to the dominance of *Vata Dosha*
- Pricking type of pain in the abdomen
 - Distension of the abdomen
 - Various kinds of discomfort which occurs due to the vitiation of *Vata Dosha*
 - No downward movement of flatus and faecal matter
 - Stiffness of the body parts
 - Confusion
 - Body ache

Rasashesha Ajirna

रसशेषेऽन्नविद्वेषो हृदयाशुद्धिगौरवे।

Ma.Ni. - 6/13

रस शब्देन रसवानाहारोऽभिप्रेतो लक्षणया, तेन रसशब्देन रसवानाहारोऽभिधीयते, तस्य शेषोऽपरिणतिलक्षणो रसशेष इति जेज्जटः।

उद्गारशुद्धावपि भक्ताकांक्षा न जायते हृद् गुरुता च यस्य।

सु.सू. - 46/

रसावशेषेण तु सप्रसेकं चतुर्थमेतत्प्रवदन्यजीर्णम्॥

उद्गारेऽपि विशुद्धतामुपगते कांक्षा न भक्तादिषु स्निग्धत्वं वदनस्य सन्धिषु रुजा कृत्वा शिरोगौरवम्।

मन्दाजीर्णसे तु लक्षणमिदं तत्रातिवृद्धे पुनः हल्लासज्वरमूर्च्छनादि च भवेत् सर्वाभयक्षोभणम्॥

आमादित्रयजमन्नज, रसशेषरसाहाररसजः; Madhu, Ma.Ni. - 6/5-6

While discussing about the concept of *Rasashesha*, in two different understanding of *Rasashesha* is available. In one context, part of the impartially formed *Rasa* remain in *Mahasrotas* (alimentary canal) and this will either stick to the alimentary canal or move either upwards or downwards. In another context, part of impartially digested *Ahara Rasa* will enter the *Rasa*. As per *Madhukosha* commentary, *Ama*, *Vidagdha* and *Vishtabha Ajirna* are *Annaja* (resulted from improperly digested food articles) whereas *Rasashesha Ajirna* are considered to be *Ahara Rasajanya*.

- Aversion to take the food substances
- Discomfort in the chest
- Heaviness of the head
- Heaviness of the body parts

Dinapaki Ajirna: The term *Dinapaki Ajirna* refers to the state of indigestion which persists in an individual for more than one day.

Prakrita Ajirna: The term *Prakrita Ajirna* is observed in each and every individual temporarily after the consumption of food.

Upadrava of Ajirna

मूर्च्छा प्रलापो वमथुः प्रसेकः सदनं भ्रमः।

Ma.Ni. - 6/13

उपद्रवा भवन्त्येते मरणं चाप्यजीर्णतः॥१३॥

- Murcha**²⁶ - altered state of consciousness
- Pralapa**²⁷ - irrelevant and incomprehensible speech
- Vamathu**²⁸ - vomiting
- Praseka**²⁹ - excessive salivation
- Sadana**³⁰ - sluggish movement of body parts

- 26 This symptom is usually attributable to the consequence of poor nutrient absorption from the intestine such as hypoglycemia or hyponatremia or hypokalemia.
- 27 This is usually attributable to consequence of poor nutrient absorption such as hypoglycemia or electrolyte imbalances.
- 28 This is attributable to the presence of excess amount food in the stomach which may lead to the stretching of stomach wall leading to closure of pyloric antrum, reverse peristaltic movements leading to vomiting.
- 29 This is attributable to the over stimulation of parasympathetic nervous system. This will lead to excess salivation owing to stimulation of salivary glands. Over stimulation of parasympathetic nervous system can even causes the excess stimulation of gastric gland to secrete the digestive juices which further dilute the ingested food leading to indigestion.
- 30 This is attributable to the presence of macromolecules in the alimentary canal. This is mainly observed in the trunk region initially.

❖ **Bhrama**³¹ – giddiness

❖ **Marana**³² – death

Consequences of Amajirna, Vishtabdhajirna and Vidagdhajirna

अजीर्णमामं विष्टब्धं विदग्धं च यदीरितम्।

विसूच्यलसकौ तस्माद्भवेच्चापि विलम्बिका॥15॥ Ma.Ni. – 6/15

- ❖ If an individual is not receiving any appropriate treatment or unwilling to receive the treatment, then it may lead to following consequences.
- ❖ If *Amajirna* is not treated properly, it may lead to the manifestation of *Visuchika*.
- ❖ If *Vishtabdhajirna* is not treated properly, it may lead to the manifestation of *Alasaka*.
- ❖ If *Vidagdhajirna* is not treated properly, it may lead to the manifestation of *Vilambika*³³.

Anaha, Adhmana and Atopa

Introduction

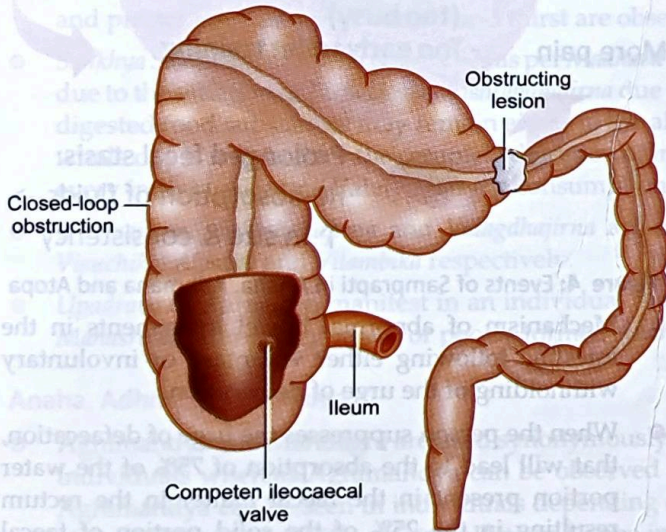


Figure 3: Pratyatma Lakshana of Anaha, Adhmana and Atopa

- ❖ *Anaha*, *Adhmana* and *Atopa* are the terms indicating spectra of the diseases where in we can observe the *Apana Vata* affliction at the beginning.
- ❖ These diseases involve *Purishavaha* and *Annavaha Srotas*. When we speak in terms of the progression

of the pathogenesis, we can say *Anaha*, *Adhmana* and *Atopa* are sequential pathological events occur in an individual.

- ❖ These are not separate disease entities. As far as specific etiological factors, premonitory symptoms and prognosis pertaining to *Anaha*, *Adhmana* and *Atopa* are not mentioned in *Ayurveda* literatures.
- ❖ In the present chapter, we are going to discuss the concept of *Anaha*, *Adhmana* and *Atopa* based on the *Lakshana* mentioned in *Ayurveda*.

Anaha

यस्य वातः प्रकुपितः कुक्षिमाश्रित्य तिष्ठति।

नाधो व्रजति नाप्यूर्ध्वमानाहस्तस्य जायते॥

Ca.Su. – 18/32, Shabda Kalpa Druma

आमं शकृद्वा निचितं क्रमेण भूयो विवद्धं विगुणानिलेन।

प्रवर्तमानं न यथास्वमेनं विकारमानाहमुदाहरन्ति॥17॥

Ma.Ni. – 27/17

Gradual accumulation of *Ama* or stool in the *Mahasrotas* (alimentary canal). This is followed by reverse movement of *Vata* and due to this, the normal course of the movement in the *Mahasrotas* i.e., *Pranavata*, *Samanavata* and *Apanavata* is getting reversed. This is known as *Anaha*.

Types of Anaha

This is of 2 types.

1. *Amaja Anaha*
2. *Purishaja Anaha*

Amaja Anaha

तस्मिन् भवन्त्यामसमुद्भवे तु तृष्णाप्रतिश्यायशिरोविदाहाः।

आमाशये शूलमथो गुरुत्वं हस्तम्भ उद्गारविघातनं च॥18॥

Ma.Ni. – 27/18

Thirst, running nose, burning sensation in the head, pricking type of pain in *Amashaya*, heaviness of the abdomen, stiffness of the chest and absence of belching are the clinical features of *Amaja Anaha*³⁴.

Purishaja Anaha

स्तम्भः कटी पृष्ठपुरीषमूत्रे शूलोऽथ मूर्च्छा शकृतश्च छर्दिः।

G
K

- 31 This could be attributable to the hampered nutrient absorption in the alimentary canal such as hypoglycemia or hyponatremia or hypokalemia.
- 32 This is attributable to hampered reabsorption of water, glucose, sodium or potassium ions. Normally, out of 6 to 7 litres of fluid either by ingestion or by secretion of digestive juices into the gastrointestinal tract, only around 75 ml of the fluid will be expelled through anus. When the person is having deficient fluid absorption, it may lead to loss of sodium also. Normally, 25 to 35 gms of sodium will be absorbed each day from the large intestine which may be decreased in case of indigestion. This may be fatal within hours, sometimes leading to death also.
- 33 The descriptions of *Visuchika*, *Alasaka* and *Vilambika* are seen in respective chapters.
- 34 Clinical features of pyloric obstruction can be observed in *Amaja Anaha*. In this condition, patient will have cardinal feature of vomiting at the evening time. The person may have visible peristalsis from left to right. Certain systemic symptoms such as thirst, weight loss, signs of dehydration can be observed.

शोथश्च पक्वाशयजे भवन्ति तथाऽलसोकानि च लक्षणानि॥११॥

Ma.Ni. - 27/ 18

Stiffness of waist, lower back, obstruction of *Mutra* and *Purisha*, pricking type of pain in the abdomen, altered state of consciousness, feculent vomiting, swelling of *Pakvashaya* and clinical features of *Alasaka*³⁵ are seen in *Purishaja Anaha*³⁶.

Adhmana

साटोप मत्पुग्रजमाध्मातमुदरं भृशम्॥

आध्मानमिति जानीयात् घोरं वातनिरोधजम्॥१८८॥ Su.Ni. - 1/88

The abdomen is excessively distended, associated with gurgling sound in the abdomen and severe pain in the abdomen. This is manifested in the individual due to the suppression of *flatus*³⁷.

Atopa

उदरे गुडगुडशब्दः वातजन्यः श्रूयत इत्यर्थः॥ च.नि.-८/६, चक्र

आटोपः उदरापूरः॥ सु.उ. - ४२/८, डल्हण

नाभेरधोमन्दसंचार वायुता॥ अ.सं.नि. - ११/४७, इन्दु

आटोपमीषत्सशब्दमाध्मानम्॥ सु.सू. १५/१५, चक्र

आटोपमाध्मानं, क्षोभमित्यन्ये॥ सु.सू. १५/१५, डल्हण

आटोपः संक्षोभः॥ सु.नि. - ९/२०, डल्हण

*Atopa*³⁸ is the clinical condition which manifested by the vitiation of *Vata Dosha* in the body. This is associated with severe abdominal pain and gurgling sound in the abdominal region. It is associated with poor movement of *Vata* in the abdominal region below the umbilicus in particular.

Samprapti Ghataka

❖ **Dosha : Vata**

❖ **Dushya : Purisha**

35 कुक्षिरानह्यतेऽत्यर्थं प्रताप्येत्परिकूजति। निरुद्धो मारुतश्चैव कुक्ष्यावुपरि धावति॥११॥

वातवचीनिरोधश्च यस्यात्यर्थं भवेदपि। तस्यालसकमाचष्टे तृष्णोद्धारौ च यस्य॥२०॥

Distension of the abdomen, darkness in front of the eyes, cooing sounds made by the patient, constipation, reverse movement of *Vata Dosha* or reverse peristaltic movement, non passage of flatus and feces, thirst and lack of belching are the Lakshana of *Alasaka*.

36 Clinical features of large intestinal obstruction will have the features of *Purishaja Anaha*. In this condition, the patient will have fullness of the flanks, pain abdomen and faeculent vomiting as a late clinical feature.

37 When the suppression of urge of passing flatus is done by an individual, this will be responsible for the absorption of 60-90% of water in the faecal matter. This leads to the formation of scybalous stool mass. This is responsible for increased frequency of loud and harsh peristaltic sounds. This will be associated with severe abdominal pain as this will be leading to the partial large intestinal obstruction.

Thus *Atopa* will follow *Adhmana*.

38 Normal peristaltic sounds consists of clicks and gurgles, occurring at a frequency of 5 to 34 per minute. When the person is presented with chronic large intestinal obstruction will have long prolonged gurgles of hyperperistalsis. These types of sounds of hyperperistalsis is termed as borborygmi.

❖ **Srotas : Purishavaha, Annavaha Srotas**

❖ **Srotodushti Prakara: Sanga and Vimargagamana**

❖ **Agni : Jatharagni affliction**

❖ **Ama : Jatharagnijanya**

❖ **Udbhava Sthana : Pakvashaya**

❖ **Sanchara Sthana : Koshtha**

❖ **Vyakta Sthana : Kukshi**

❖ **Rogamarga : Abhyantara**

❖ **Vyadhyavastha : Ashukari or Chirakari**

❖ **Sadhyasadhyata : Sadhya to Yapyia** depending upon the duration of illness, involvement of *Dushya* in the manifestation

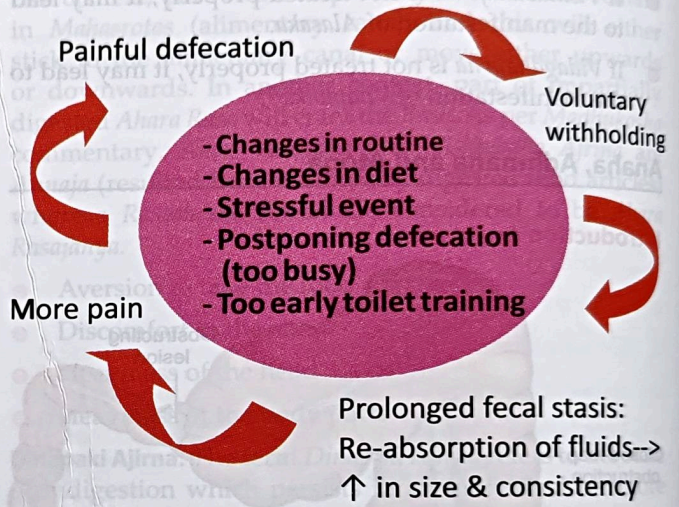


Figure 4: Events of Samprapti in Anaha, Adhmana and Atopa

❖ Mechanism of abnormal bowel movements in the intestine following either voluntary or involuntary withholding of the urge of defaecation.

❖ When the person suppresses the urge of defaecation, that will lead to the absorption of 75% of the water portion present in the faecal matter in the rectum resulting in the 25% of the solid portion of faecal matter which becomes even more solidified.

- This in turn leads to the another urge of defaecation to occur after several hours of bowel movements resulting in the increased peristaltic movements in the large intestine to facilitate the expulsion of faecal matter.
- The subsequent suppression of urge of defaecation will lead to stretching of the rectum and later that will lead to the distension of the sigmoid colon.
- When an individual suppresses the urge of defaecation, that will lead to the abnormal increase

of *Apanavata* in the *Mahasrotas*, that will lead to the absorption of *Dravata* of *Purisha* leading to hardness of *Purisha* and later there will be reverse movement of *Apanavata* (increase of *Ruksha* quality in the *Pakvashaya*), which is termed as *Udavarta*.

Thus, initially, there will be appearance of *Anaha*, which will be followed by *Adhmana* and later, there will be *Atopa* and afterwards, these may interchange depending upon the nature of pathological events taking place in the patient's body.

Key points of the chapter

Agnimandya- Ajirna

न जीर्यति सुखेनात्र विकारान् कुरुतेऽपि च। तदजीर्णमिति प्राहुस्तन्मूला विविधा रुजः॥

Gananath Sen

- As per veteran *Ayurveda* scholar, Gananath Sen, *Ajirna* is a condition where a person cannot able to digest the ingested food and is responsible for various kinds of discomfort and various kinds of disease manifestation.

Jirnahara Lakshana

उद्गारशुद्धिरुत्साहो वेगोत्सर्गो यथोचितः। लघुता क्षुत्पिपासा च जीर्णाहारस्य लक्षणम्॥2.4॥

Ma.Ni. - 6/24

- Clear eructation without taste perception; able to do any kind of activities; proper evacuation of different Vega i.e. Vata, Vit and Mutra (flatus, faecal matter and urine respectively) at proper time; lightness of the alimentary canal and proper appearance of hunger and thirst are observed in an individual who has proper digestion of food.
- *Sankhya Samprapti* of *Ajirna* is six types as per *Madhava Nidana* – *Amajirna* due to vitiation of *Kapha Dosha*, *Vidagdhajirna* due to the vitiation of *Pitta Dosha*, *Vishtabdhajirna* due to the vitiation of *Vata Dosha*, *Rasasheshajirna* where in partially digested food substances may remain either in the alimentary canal or in the *Rasa Dhatu*, *Dinapaki Ajirna* where in whole day is required for the complete digestion of ingested food and *Prakrita Ajirna* which is observed in each and every individual immediately after the consumption of food for a certain period of time.
- If *Amajirna*, *Vishtabdhajirna* and *Vidagdhajirna* are not treated properly, they may lead to the manifestation of *Visuchika*, *Alasaka* and *Vilambika* respectively.
- *Upadrava* of *Ajirna* can manifest in an individual due to poor absorption or non-absorption of *Rasa* formed in the *Mahasrotas* (alimentary canal) or poorly formed *Ahara Rasa* getting into the *Rasa Dhatu* for circulation.

Anaha, Adhmana and Atopa

- *Agnimandya* and *Mandagni* are used synonymously most of the times. *Mandagni* is usually seen in *Kapha Prakriti* individuals whereas *Agnimandya* can be observed in individuals irrespective of the *Prakriti* of the individuals. *Agnimandya* can be seen in individuals depending on the exposure to the aetiological factors, which leads to the abnormal increase of either *Vata* or *Pitta* or *Kapha* in the body.
- *Ahara Parinama Krama* is explained in brief in this chapter. *Pranavata* will lead to the movement of food substances from the mouth to the *Amashaya*. In the *Amashaya*, ingested food stays for a certain period of time. *Ushma*, *Kleda*, *Sneha* will act on the ingested food. After a certain period of time, partially digested food substances will be taken to the distal parts by *Samana Vata* which is responsible for the separation of nutrients and metabolites. Further, *Samana Vata* helps in the absorption of nutrients and expulsion of metabolites (undigested food residues) further into *Pakvashaya*. *Apana Vata* will take up the function of expulsion of undigested food residues to the exterior in due course of time.
- *Agnimandya* and *Ajirna* are manifested in the individuals as abnormal cycles. *Agnimandya* will lead to the occurrence of *Ajirna* and *Ajirna* will lead to the manifestation of *Agnimandya* and the cycle continues.
- *Kashyapa* mentioned the aetiological factors of *Agnimandya* in the context of *Amlapitta* in *Khila Sthana* of *Kashyapa Samhita*.

न जीर्यति सुखेनात्र विकारान् कुरुतेऽपि च। तदजीर्णमिति प्राहुस्तन्मूला विविधा रुजः॥

Gananath Sen

- As per veteran *Ayurveda* scholar, Gananath Sen, *Ajirna* is a condition where a person cannot able to digest the ingested food and is responsible for various kinds of discomfort and various kinds of disease manifestation.

Jirnahara Lakshana

उद्गारशुद्धिरुत्साहो वेगोत्सर्गी यथोचितः। लघुता क्षुत्पिपासा च जीर्णाहारस्य लक्षणम्॥२४॥

Ma.Ni. - 6/24

- ⊗ Clear eructation without taste perception; able to do any kind of activities; proper evacuation of different Vega i.e. Vata, Vit and Mutra (flatus, faecal matter and urine respectively) at proper time; lightness of the alimentary canal and proper appearance of hunger and thirst are observed in an individual who has proper digestion of food.
- ⊗ Sankhya Samprapti of Ajirna is six types as per Madhava Nidana – Amajirna due to vitiation of Kapha Dosha, Vidagdhajirna due to the vitiation of Pitta Dosha, Vishtabdhajirna due to the vitiation of Vata Dosha, Rasasheshajirna where in partially digested food substances may remain either in the alimentary canal or in the Rasa Dhatu, Dinapaki Ajirna where in whole day is required for the complete digestion of ingested food and Prakrita Ajirna which is observed in each and every individual immediately after the consumption of food for a certain period of time.
- ⊗ If Amajirna, Vishtabdhajirna and Vidagdhajirna are not treated properly, they may lead to the manifestation of Visuchika, Alasaka and Vilambika respectively.
- ⊗ Upadrava of Ajirna can manifest in an individual due to poor absorption or non-absorption of Rasa formed in the Mahasrotas (alimentary canal) or poorly formed Ahara Rasa getting into the Rasa Dhatu for circulation.

Self-Assessment: Questions for Review**Long answer questions**

- ⊗ Define Ajirna. Explain the Nidana, Bheda, Samanya Lakshana, Upadrava of Ajirna
- ⊗ Explain the Samprapti of Agnimandya and describe how it will lead to the manifestation of Ajirna

Short answer questions

- ⊗ Explain the differences between Anaha, Adhmana and Atopa
- ⊗ Explain Amajirna
- ⊗ Explain Jirnahara Lakshana and Samanya Ajirna Lakshana
- ⊗ Explain the differences between Agnimandya and Mandagni
- ⊗ Explain the Sankhya Samprapti of Ajirna
- ⊗ Explain the types of Anaha

MCQs**1. Agnimandya is seen in**

- a. Vata Prakriti individuals;
- b. Pitta Prakriti individuals;
- c. Kapha Prakriti individuals;
- d. Any of the above e. All of the above

Answer: (d)

2. Mandagni is seen in

- a. Vata Prakriti individuals;
- b. Pitta Prakriti individuals;
- c. Kapha Prakriti individuals;
- d. None of the above

Answer: (c)

3. The excess consumption of Kulattha (horse gram) will lead to the abnormal increase of

- a. Pitta;
- b. Vata;

c. Kapha;

d. Vata Pitta and Kapha

Answer: (a)

4. Alcoholic beverages will lead to the vitiation of

- a. Vata;
- b. Pitta;
- c. Kapha;
- d. All of the above

Answer: (b)

5. How many types of Viruddha are mentioned by Charaka?

- a. 12;
- b. 15;
- c. 18;
- d. 24

Answer: (c)

6. Vishtabdhajirna is manifested due to the vitiation of _____ Dosha.

- a. Vata;
- b. Pitta;
- c. Kapha;
- d. All of the above

Answer: (a)

7. Sankhya Samprapti of Ajirna as per Madhavakara is

- a. 4;
- b. 5;
- c. 6;
- d. 3

Answer: (c)

8. The type of Ajirna which requires whole day to have digestion of ingested food is termed as

- a. Amajirna;
- b. Rasasheshajirna;
- c. Prakrita Ajirna;
- d. Dinapaki Ajirna

Answer: (d)

9. Visuchika is manifested in an individual due to the consequence of

- a. Vishtabdhajirna;
- b. Amajirna;
- c. Rasasheshajirna;
- d. Vidagdhajirna

Answer: (b)

10. As per Madhukosha commentary, Ajirna is considered to be

- a. Kaphajanya;
- b. Tridoshajanya;

- c. Vatajanya; d. Pittajanya
Answer: (b)

11. The Vata vitiated in Adhmana is

- a. Pranavata; b. Udanavata;
c. Vyanavata; d. Apanavata

Answer: (d)

12. Types of Anaha are

- a. 2; b. 3;
c. 4; d. 7

Answer: (a)

13. Amaja and Purishaja are the types of

- a. Anaha; b. Adhmana;
c. Atopa; d. Pratyadhmana

Answer: (a)

14. Gudagudayana Shabda is the characteristic feature of

- a. Anaha; b. Adhmana;
c. Atopa; d. Pratyadhmana

Answer: (c)

15. Dosha involved in Adhmana is

- a. Vata; b. Pitta;
c. Kapha; d. Sannipata

Answer: (a)

Chapter resources

- ✿ Kashyapa Samhita Khila Sthana – 16th chapter, Amlapitta Chikitsa Adhyaya
- ✿ Charaka Samhita, Sutra Sthana – 26th chapter, Atreya Bhadrakapya Adhyaya
- ✿ Charaka Samhita, Vimana Sthana – 1st chapter, Rasa Vimana Adhyaya
- ✿ Charaka Samhita, Chikitsa Sthana – 15th chapter, Grahani Dosha Chikitsa Adhyaya
- ✿ Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- ✿ Madhava Nidana – 6th chapter: Agnimandya Ajirna Visuchika Alasaka Vilambika Nidana
- ✿ Madhukosha commentary, Madhava Nidana, - 6th chapter: Agnimandya Ajirna Visuchika Alasaka Vilambika Nidana
- ✿ Sushruta Samhita Sutrasthana – 46th chapter: Annapana Vidhi Adhyaya
- ✿ Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- ✿ www.karger.com related to excessive yawning and disturbances in digestion
- ✿ Madhava Nidana – 27th chapter – Udavarta Anaha Nidana Adhyaya
- ✿ Sushruta Samhita, Nidana Sthana – 1st chapter – Vatavyadhi Nidana Adhyaya
- ✿ Charaka Samhita, Nidana Sthana – 8th chapter – Apasmara Nidana Adhyaya
- ✿ Sushruta Samhita, Uttara Tantra – 42nd chapter – Gulma Pratishedha Adhyaya
- ✿ Sushruta Samhita, Sutra Sthana – 15th chapter – Dosha Dhatu Mala Kshaya Vridhhi Vijnaniya Adhyaya
- ✿ Sushruta Samhita Nidana Sthana – 9th chapter – Vidradhi Nidana Adhyaya
- ✿ Ashtanga Sangraha Nidana Sthana – 11th chapter – Vidradhi Vridhhi Gulma Nidana Adhyaya
- ✿ Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- ✿ The Merck Manual of Diagnosis and Therapy by Robert S Porter and Justin L Kaplan, 19th edition, Merck Sharp & Dohme Corp, New Jersey, 2011
- ✿ Chamberlain's symptoms and signs in clinical medicine by Colin Ogilvie and Christopher C Evans, 12th edition, Butterworth-Heinemann Publishers, 1997
- ✿ Best & Taylor's physiological basis of medical practice by O P Tandon and Y Tripathi, 13th edition, Wolters Kluwer Health (India) Pvt. Ltd, Gurgaon, 2012
- ✿ Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006
- ✿ www.intechopen.com
